

# METROPOLITAN GENERAL HOSPITAL

## Inpatient Daily Progress Notes — General Medicine Ward 4B

|            |                                                   |            |                    |
|------------|---------------------------------------------------|------------|--------------------|
| Patient:   | John Michael Carter                               | MRN:       | MGH-2024-087432    |
| DOB:       | 14 March 1968 (Age: 57)                           | Attending: | Dr. Priya Nair, MD |
| Diagnosis: | Acute Decompensated Heart Failure (HFrEF, EF 35%) |            |                    |

### Day 1 — 03 June 2026 | 18:00

Author: Dr. Arjun Mehta, MD (PGY-3) | Reviewed by: Dr. Priya Nair, MD

#### Subjective

Patient reports mild improvement in dyspnoea since initiation of IV diuretics. Still requires 2–3 pillows to sleep. Denies chest pain or palpitations. Tolerating clear liquids; appetite decreased. Anxious about his condition.

#### Objective

|         |                                                                                                     |
|---------|-----------------------------------------------------------------------------------------------------|
| Vitals: | BP 148/90   HR 92   RR 20   SpO2 94% (2L NC)   Temp 36.7°C   Wt 93.2 kg (↓0.8 kg)                   |
| I/O:    | Input: 1,240 mL   Output: 2,100 mL urine (net –860 mL)   No emesis                                  |
| Exam:   | Reduced crackles at right base. Mild JVD remains. Lower extremity oedema 2+ bilaterally, unchanged. |
| Labs:   | K+ 3.0 (L) — KCl replacement ordered. Cr 1.9 (stable). BNP pending.                                 |

#### Assessment

Acute decompensated HFrEF responding to IV diuresis — adequate urine output. Hypokalaemia secondary to furosemide — requires potassium supplementation. No arrhythmia on telemetry.

#### Plan

- Continue IV furosemide 80 mg BID; reassess fluid balance q8h.
- KCl 40 mEq PO x 2 doses for K+ 3.0; recheck K+ in AM.
- Restrict fluid to 1.5 L/day; sodium < 2 g/day; daily weights.
- Cardiology review requested for AM rounds.
- Repeat CXR in 24 hours.
- Reassure patient and family — explain treatment goals.

### Day 2 — 04 June 2026 | 08:30

Author: Dr. Arjun Mehta, MD (PGY-3) | Cardiology: Dr. Neha Gupta, MD

#### Subjective

Patient reports significantly improved breathing. Able to sleep flat with 1 pillow. Mild fatigue. Passing good volumes of urine overnight. Asks about discharge timeline.

#### Objective

|         |                                                                                                    |
|---------|----------------------------------------------------------------------------------------------------|
| Vitals: | BP 138/86   HR 84   RR 16   SpO2 97% (1L NC)   Temp 36.5°C   Wt 91.4 kg (↓1.8 kg)                  |
| I/O:    | 24h Input: 1,100 mL   24h Output: 3,200 mL (net –2.1 L)   Total net since admission: –2.96 L       |
| Exam:   | Clear breath sounds bilaterally. No crackles. JVD resolved. Oedema reduced to 1+ bilateral ankles. |
| Labs:   | Na+ 138   K+ 3.7 (normal after replacement)   Cr 1.7 (↓from 1.9)   NT-proBNP 2,650 (↓from 4,850)   |

**CXR:** Interval improvement in pulmonary vascular congestion. Reduced pleural effusions (small bilateral).

**Echo (bedside):** EF 33% — no significant change from prior. Mild mitral regurgitation. LV dilated.

### Assessment

Good clinical response to IV diuresis — net negative fluid balance, improved oxygenation, resolved JVD, reduced oedema. Improving renal function. NT-proBNP trending down. Cardiology reviewing for optimisation of GDMT.

### Plan

1. Transition from IV to PO furosemide 80 mg BID (double home dose).
2. Add sacubitril/valsartan 24/26 mg BID per cardiology recommendation (start low, titrate).
3. Continue carvedilol 12.5 mg BID; lisinopril discontinued (switch to ARNI).
4. Uptitrate spironolactone to 50 mg/day after discharge.
5. Metformin resumed (Cr improving).
6. Referral to heart failure clinic for outpatient follow-up in 7–10 days.
7. Dietitian referral — low-sodium diet counselling.
8. Discharge planning initiated for Day 3 if vitals and labs stable.

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## Day 3 — 05 June 2026 | 09:00

*Author: Dr. Priya Nair, MD (Attending)*

### Subjective

Patient feels 'much better.' Minimal dyspnea at rest, mild on exertion. Sleeping flat. Eager to go home. Understands importance of diet and medication adherence after education session with nursing team.

### Objective

**Vitals:** BP 132/82 | HR 78 | RR 14 | SpO2 98% (RA) | Temp 36.6°C | Wt 89.8 kg (↓1.6 kg, −4.2 kg total)

**I/O:** 24h Input: 1,050 mL | 24h Output: 2,400 mL (net −1.35 L)

**Exam:** No crackles or wheeze. Normal S1/S2, no S3 today. No JVD. Trace ankle oedema only.

**Labs:** Na+ 140 | K+ 4.1 | Cr 1.5 (near baseline 1.4) | Glucose 108 | NT-proBNP 1,240 (↓↓)

### Assessment

Clinically euvolaemic. Oxygenation restored on room air. Renal function returned to near-baseline. NT-proBNP significantly improved. Haemodynamically stable. **Patient is medically fit for discharge today.**

### Plan

1. DISCHARGE today — discharge summary to be completed by 12:00.
  2. Discharge medications: PO furosemide 80 mg daily, sacubitril/valsartan 24/26 mg BID, carvedilol 12.5 mg BID, spironolactone 50 mg daily, atorvastatin 40 mg nocte, aspirin 81 mg daily, metformin 500 mg BID.
  3. Follow-up: Heart Failure Clinic — 12 June 2026. PCP — 10 June 2026.
  4. Return precautions: return to ED if weight increases > 2 kg in 2 days, worsening SOB, or chest pain.
  5. Patient education completed: sodium restriction, fluid restriction, daily weights, medication compliance.
  6. Discharge scripts printed. Family briefed.
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